

Proposed Prescribing Information
For the use of a Registered Medical Practitioner or a Hospital or a Laboratory

ALFUTOR ER TABLETS 10 MG
Alfuzosin Hydrochloride Extended Release Tablets USP 10 mg

BRAND OR PRODUCT NAME

ALFUTOR ER TABLETS 10 mg

NAME AND STRENGTH OF ACTIVE SUBSTANCE(S)

Each uncoated extended release tablet contains:

Alfuzosin hydrochloride USP10 mg

PRODUCT DESCRIPTION

White to off white, round shape, flat face beveled edge tablets debossed with '1021' on one side and '10' on other side.

DOSAGE FORM

Extended release tablet.

PHARMACODYNAMICS / PHARMACOKINETICS

Pharmacodynamics

Pharmacotherapeutic group: alpha-adrenoreceptor antagonists

ATC code: G04CA01

Alfuzosin is an orally active quinazoline derivative. It is a selective, peripherally acting antagonist of postsynaptic alpha-1-adrenoceptors.

Pharmacokinetics

The binding of alfuzosin to plasma proteins is about 90%. Alfuzosin undergoes extensive metabolism by the liver, with only 11 % of the parent compound being excreted unchanged in the urine. The majority of the metabolites (which are inactive) are excreted in the faeces (75 to 91 %). Metabolic interactions: CYP3A4 is the main hepatic enzyme isoform involved in the metabolism of alfuzosin.

INDICATION

Treatment of the functional symptoms of benign prostatic hypertrophy (BPH).

Adjuvant Treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy.

RECOMMENDED DOSAGE AND MODE OF ADMINISTRATION

Oral Use:

The tablet must be swallowed whole with a glass of water. The recommended dosage is one 10 mg tablet per day, to be taken immediately after the evening meal.

Adjuvant Treatment to a catheter in acute urinary retention related to benign prostatic hypertrophy:

The recommended dosage is one 10 mg tablet per day, to be taken after a meal, from the first day of catheterization onwards. The treatment is administered for 3 to 4 days, i.e. 2 to 3 days while the catheter is in place and one day after it is removed.

CONTRAINDICATIONS

This medicinal product must not be administered in the following situation :

- hypersensitivity to alfuzosin and/or any of the other ingredients
- postural hypotension
- liver failure
- severe kidney failure (creatinine clearance <30ml/min)
- in combination with ritonavir-boosted protease inhibitors
- in combination with telaprevir
- in combination with other alpha-1 receptor blockers

WARNINGS AND PRECAUTIONS

As with all alpha-1-blockers in some subjects, in particular patients receiving antihypertensive medications or nitrates, postural hypotension with or without symptoms (dizziness, fatigue, sweating) may develop within a few hours following administration. In such cases, the patient should lie down until the symptoms have completely disappeared.

These effects are transient, occur at the beginning of treatment and do not usually prevent the continuation of treatment. The patient should be warned of the possible occurrence of such events.

As with all alpha1-receptor blockers, alfuzosin should be used with caution in patients with acute cardiac failure.

Care should be taken when Alfuzosin ER is administered to patients who have had a pronounced hypotensive response to another alpha-1-blocker.

Treatment should be initiated gradually in patients with hypersensitivity to alpha-1-blockers. Alfuzosin ER should be administered carefully to patients being treated with antihypertensives. Blood pressure should be monitored regularly, especially at the beginning of treatment.

Patients with congenital QTc prolongation, with a known history of acquired QTc prolongation or who are taking drugs known to increase the QTc interval should be evaluated before and during the administration of alfuzosin.

In coronary patients, the specific treatment for coronary insufficiency should be continued. If angina pectoris reappears or worsens Alfuzosin ER should be discontinued.

As there are no clinical safety data available in patients with severe renal impairment (creatinine clearance < 30ml/min), alfuzosin 10 mg Extended released tablets should not be administered to this patient group.

Patients should be warned that the tablet should be swallowed whole. Any other mode of administration, such as crunching, crushing, chewing, grinding or pounding to powder should be prohibited. These actions may lead to inappropriate release and absorption of the drug and therefore possible early adverse reactions.

The 'Intraoperative Floppy Iris Syndrome' (IFIS, a variant of small pupil syndrome) has been reported during cataract surgery in some patients on or previously treated with alpha-1-blockers. Although the risk of this event with alfuzosin appears very low, ophthalmic surgeons should be informed in advance of cataract surgery of current or past use of alpha-1-blockers, as IFIS may lead to increased procedural complications. The ophthalmologists should be prepared for possible modifications to their surgical technique.

Effects on ability to drive and use machines

There are no data available on the effect on driving vehicles. Adverse reactions such as vertigo, dizziness and asthenia may occur essentially at the beginning of treatment. This has to be taken into account when driving vehicles and operating machinery .

INTERACTIONS WITH OTHER MEDICAMENTS

Contraindicated combination

Ritonavir-boosted protease inhibitor, telaprevir: Risk of increased plasma alfuzosin concentrations and increased undesirable effects.

Inadvisable combinations

- Antihypertensive alpha receptor blockers: Enhanced hypotensive effect. Risk of severe postural hypotension.
- Ketoconazole, itraconazole: Enhanced hypotensive effect. Risk of severe postural hypotension.
- Clarithromycin, erythromycin Risk of increased plasma alfuzosin concentrations and increased undesirable effects.
- Vardenafil Risk of postural hypotension, particularly in elderly subjects.

Combination requiring precautions for use

- Phosphodiesterase type 5 inhibitors (sildenafil, tadalafil) Risk of postural hypotension, particularly in elderly subjects. Treatment should be initiated at the lowest recommended dose and adjusted gradually if necessary.

Administration of general anaesthetics in patients treated with alfuzosin may contribute to blood pressure instability.

Combinations to be taken into consideration

- Antihypertensives except alpha-receptor blockers, Enhanced hypotensive effect. Higher risk of postural hypotension.
- Nitrates, nitrites and related drugs (isosorbide dinitrate, isosorbide, linsidomine, molsidomine, nicorandil, nitroglycerin) Increased risk of hypotension, particularly postural.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Due to the type of indication this section is not applicable

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

Following adverse effects were reported: -

Nervous system disorders

Common: faintness/dizziness, headache

Uncommon: syncope, vertigo, malaise, drowsiness

Eye disorders

Uncommon: vision abnormal

Not known: intraoperative floppy iris syndrome

Cardiac disorders

Uncommon: tachycardia, palpitations, hypotension (postural),

Very rare: New onset, aggravation or recurrence of angina pectoris in patients with pre-existing coronary artery disease.

Not known: atrial fibrillation

Vascular disorders

Uncommon: hypotension (postural), flushing

Blood and lymphatic system disorders

Not known: neutropenia, thrombocytopenia

Respiratory, thoracic and mediastinal disorders

Uncommon: rhinitis

Gastro-intestinal disorders

Common: nausea, abdominal pain

Uncommon: diarrhoea, dry mouth, vomiting

Not known: vomiting

Hepatobiliary disorders

Frequency unknown: hepatocellular injury, cholestatic liver disease.

Skin and subcutaneous tissue disorders

Uncommon: rash, pruritus

Very rare: urticaria, angioedema

Reproductive system and breast disorders

Frequency unknown: priapism

General disorders and administration site conditions

Common: asthenia

Uncommon: flushes, oedema, chest pain

OVERDOSE AND TREATMENT

In case of overdosage, the patient should be hospitalised, kept in the supine position, and conventional treatment of hypotension should take place.

In case of significant hypotension, the appropriate corrective treatment may be a vasoconstrictor that acts directly on vascular muscle fibres.

Alfuzosin is not dialysable because of its high degree of protein binding.

ACTIVE INGREDIENT AND INACTIVE INGREDIENT:

Active Ingredient:

Alfuzosin Hydrochloride

Inactive Ingredient:

Lactose Anhydrous
Hypromellose K100M CR
Povidone (K30)
Colloidal Silicone Dioxide
Isopropyl Alcohol
Povidone K90D (40 mesh)
Magnesium Stearate
Talc

STORAGE CONDITIONS

Store below 30°C. Protect from moisture and light.

PACKAGING AVAILABLE

Alfutor ER Tablets 10 mg is packed in Cold forming Alu-blister (Opa/Alu/PVC-Alu) of 10 tablets.
Each blister strip contains 10 tablets. 3 blister strips are packed in a carton.

DATE OF REVISION OF PACKAGE INSERT

11.04.2018

NAME AND ADDRESS OF MANUFACTURER



TORRENT PHARMACEUTICALS LTD.

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